جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ابتسام زين العابدين محمد جبريل
المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** مممممممممم
رقم البطاقة : 26804220101509 **رقم التليفون :** 01266598415 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 22-04-1968 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ابتسام زين العابدين محمد جبريل
National ID: 26804220101509

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bone scintigraphy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تحقيق الشخصية



ابتسام

زين العابدين محمد جبريل

١٨ ش ٦اكتوبر - محطة مشعل

الهرم - الجيزه

٢٦٨٠٤٢٢٠١٠١٥٠٩

LB7309845

٢٦٨٠٤٢٢٠١٠١٥٠٩ ٢٠٢٥/٠٣

حاصلة على معهد فنى صناعى

انثى مسلمة أرملة

١ البطاقة سارية حتى ٢٠٣٢/٠٣/٢٢

National ID · Front& Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافروسي انترناشونال

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 4 / 1 / 2026 G

Name	ايمان مزين العابدين	MRN	٥٢٢٤٣١
		ID	٢٦٨٠٤٢٢٠١٠١٥٠٩
Age	58	Phone	٠١١٥٧١٨٦٩٨٤
Menopausal status	Post menopausal	Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	-		

Clinical Data * rt breast Cancer with oligometastasis to Mediastinal LN since 1/2022

BX: IDC G II

ER:	PR:	HER 2:	Ki67:
8/8	7/8	+3	

R L

PS - 1

breast

Sonomammography: * Known case of Cancer C $T_4N_2M_1$ at 1/2022 * initial lt Axillary LN. FDG Avid Mediastinal LN

PET/CT → rt breast Mass and Mediastinal LN Measuring 2.2 cm SUVS x 4 CYCLES

Mets Workup: * Rec AI and trastuzumab 8/2022 → bCT/PD. CT → Mediastinal LN and lapatinib for 6 cycles. CT_2 → RD and Lapatinib. last lapatinib taken

TNM

Measuring 1.4 x 0.9 s/p Docetaxel → 1/2023 = s/p exemestane in 8/2024 and Letrozole

Recommendations: exemestane alone.

* 12/2024 → PD of rt breast Mass and Axillary LN → 3/2025: s/p Capecitabine received only 2 cycles then CT → P.D of rt Axillary LN - Pt on Xeloda (2k2) tykerb from (NGO) since 1/2025 till Now. last CT CAP 9-11/2025 → HFL at labe segment III 1.6 x 1.6 cm → Rt Hepatic labe segment VI (1.5 x 1 cm) (Stable) Likely Hemangioma

MDT OUTCOME & PLAN

→ For surgery

- with Hand. feet syndrome grade 2

~~NOT seen Showed Immunotherapy~~

~~to Cont. delete with 10% dose reduction tykerb~~ re-evaluate after 3 cycles

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

[[DOCTYPE:MDT_FORM]]

Name: ابتسام زين العابدين محمد **MR. :** 2699	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient reports left arm and breast pain and has just restarted Tykerb, which was previously stopped for six months; a potential switch to TDM-1 at NGO is being considered if the current regimen fails.

Post-mastectomy left breast cancer with mediastinal lymph node metastasis (oligometastatic) and IDC G2, ER 8/8, PR 7/8, HER2 3+. Initial PET-CT showed SUV 5 in mediastinal nodes. The patient received 5 months of Herceptin and Femara, followed by Docetaxel and Tykerb (6 cycles). Tykerb was discontinued in August 2024 and the patient continued Aromasin until April 2025. A trial of Xeloda (2 × 2) was initiated, with pending SM labs; hand and foot toxicity was grade 2.

Clinical progression was noted for Xeloda 2 × 2 with Tykerb pending SM labs; bilirubin dropped to 1.32. The patient now has progressive disease of the left breast (locally advanced breast cancer) with pathological level 1-2 lymph node involvement; a bone scan is pending. A trial of TDM-1 at Mersal is being considered.

The patient is a 58-year-old postmenopausal woman. Initial biology: ER 8/8, PR 7/8, HER2 3+. Stage at diagnosis: cT4N2 with mediastinal lymph node metastasis (oligometastatic). Histology: IDC G2. Treatment history: 1st line trastuzumab + AI → local progression; 2nd line docetaxel + lapatinib → response, then maintenance AI + lapatinib; lapatinib stopped Aug 2024, AI continued to April 2025 → progression; 3rd line capecitabine (Xeloda) → stable disease initially (2 × 2 × 14) → now progression with new bone metastases (skull, femur) on April 2026 bone scan; hand-foot syndrome present. MDT recommends further therapy.

MDT decided to start Navelbine while awaiting TDM-1 from NGO. The patient has received Herceptin IV 440 mg from a separate NGO. The plan is c1 Navelbine + Herceptin.

The patient has post-mastectomy left breast cancer (cT4N2M1) with mediastinal lymph node and newly developed bone metastases, ER 8/8, PR 7/8, HER2 3+. Initially treated with Femara and Herceptin for 5 months, then taxotere and Tykerb, followed by maintenance Tykerb and Aromasin for one year until February 2025. Capecitabine was given for one year until April 2026, with progression of left axillary lymph node and new bone metastases. The patient is now receiving Navelbine and Herceptin from NGO, with approval of Kadcyla pending for cycle 2 of Navelbine 80 mg (SA 1.3) and cycle 2 of Herceptin 275 mg.

On examination, the patient has left breast induration and decreased left axillary lymph node size after cycle 3 of Navelbine 80 mg and Herceptin 275 mg, with an estimated efficacy of 62% at day 1/7. Bone care is ongoing.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

On examination, the patient has left breast mass with induration, skin dimpling, and skin nodules. Approval of Herceptin or Kadcyla from NGO is pending, along with a dental examination. The patient is scheduled for cycle 4 of Navelbine 80 mg on days 1 and 8, with new CT scans pending.

The patient started Kadcyla at Mersal, receiving cycle 1 on 10 August. A CT was performed without comparison. The patient has not yet received cycle 4 of Navelbine 80 mg on days 1 and 8 after labs. The patient has completed 1 cycle of Kadcyla and 4 cycles of Navelbine.

Patient Name	: الأستاذه / ابتسام زين العابدين محمد جبريل	Request No.	: 13002394
Patient No.	: 26804220101509	Request Date	: 03/02/2022
Path No.	: SR-71/22	Verification Date	: 08/02/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 53 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left locally advanced breast cancer

Nature of the specimen: Slide Revision

Gross Examination:
One Block (515/22) + 3 Slides

Microscopic examination:
- Histological type: Invasive duct carcinoma with mucoid features
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (2+2+2)
- Biomarkers:

ER: Strong nuclear reaction in 100% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 80% of tumor cells (Allred score 7/8)
Her-2: Strong complete membrane reaction in >10% of tumor cells (score 3+)

Diagnosis:
Left breast cancer, Slide revision:
- Invasive duct carcinoma GII with mucoid features
- ER: Positive (8/8).
- PR: Positive (7/8).
- HER-2: Positive (score 3+)

Prof. Aml Hareedy
Dr. Menna Atef

Dr. Walaa Ghonaim

14/2

Verified by :
abdelrahman ibrahim

الغرفة رقم (4) - الدور الأول - مبنى رقم (937) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 937- Nile Corniche - Misr El Qadeema- Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Ebtesam Zein Al Abedin Mohamed	Date : 05/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 55 Year(s)
Medical ID: 431/2022	Pat. ID: 15768

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- Left diffuse skin thickening with underlying trabecular thickening with UOQ amorphous microcalcifications. US showed UOQ spiculated isoechoic solid lesion measuring 4.5x1.5 cm
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed right 10:00 well-defined hypoechoic solid lesion measuring 12x6 mm
- Normal right skin thickness and nipple-areola complex.
- Right non-specific axillary LNs.
- Left single pathologically enlarged axillary lymph node seen in level I, measuring 0.7x0.6 cm

Opinion:

- Left pathologically proved locally advanced malignant mass with malignant microcalcifications with ipsilateral pathological axillary lymph node (BIRADS 6)
- Right 10:00 probably benign looking solid lesion. For follow up (BIRADS 3)

Yours truly

Signature Dr. Yasmin Nada

05/02/2024 2:58 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name: ebtesam zainelabeeden
Date: 29 December 2025

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Average in size (15cm at Rt MCL) homogenous parenchymal echo pattern with smooth surface.*
- *No definite hepatic focal lesions or intra hepatic biliary radicle dilatation.*
- *Portal vein is patent and not dilated.*

Gall Bladder: -

- Average distention and capacity with stone measuring 14mm seen inside
- The wall is not thickened.
- *CBD is not dilated.*

Spleen: -

- *Average size, homogenous.*

- **Kidneys: -**
  - *Both are average in size, shape and echogenicity*
  - *No stones or dilated pelvicalyceal system*
  - *No gross lesion*

Para-aortic and pancreatic area*: show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Average capacity with no stones or diverticular out pouching*

Uterus and adnexal: *no abnormalities could be detected*

Opinion:

Calcular GB

Best regards
Dr. NERMEEN RAMADAN FEBR. MD

Ultrasound
[[DOCTYPE:ULTRASOUND]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Ebtesam Zein Al Abedin Mohamed	Date : 09/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 57 Year(s)
Medical ID: 431/2022	Pat. ID: 15768

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Compared to previous CT report dated 30/7/2025.

I Chest:

- **Stationary course regarding the:**
  - left breast diffuse parenchymal distortion, skin thickening and multiple irregular lesions with multiple calcification foci.
  - **No appreciable changes** as regarding the multiple left axillary level I and II lymph nodes.

Rather stationary course of :

- Prominent mediastinal lymph nodes.
- Bilateral apical fibrosis.
- Right breast ill-defined rather high dense irregular lesion measuring about 16x12 mm.
- No evidence of pleural effusion or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size showing stable left hepatic lobe segment III (1.6x1 cm) and right hepatic lobe segment V hypodense focal lesion (1.5x1 cm) likely hemangiomata.
- Patent homogenously enhancing portal vein and its main tributaries.
- Still noted gall bladder stone.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTIAN

Patient Name: Ebtesam Zein Al Abedin Mohamed	Date : 09/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 57 Year(s)
Medical ID: 431/2022	Pat. ID: 15768

- No abdominal or pelvic masses or collections.
- Stationary osteopenic texture of the examined bones with innumerable lytic lesions.

Conclusion:

Follow up study showing:

- **Rather stationary course of the study.**

Signature reham kheder

12/11/2025 2:27 PM

CT Scan
[[DOCTYPE:CT_SCAN]]

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Radiology Department

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Total DLP (mGy.cm):

Patient Name: Ebtesam Zein Al Abedin Mohamed	Date : 22/07/2024
Exam : C.T CHEST	Age : 56 Year(s)
Medical ID: 431/2022	Pat. ID: 15768

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

- **Compared to previous CT report dated 20-1-2024.**

I Chest:

- **Rather stationay course of :**
  - Left breast diffuse edema pattern with skin thickness.
  - Prominent left axillary level I and II LNs.
  - Prominent mediastinal lymph nodes.
  - Left LLL posterior segment faint sub pleural reticulo noduler densities.
  - Bilteral apical fibrosis.
  - Right breast ill defined rather high dense irreguler lesion measuring about 13x12 mm.
- No evidence of pleural effusion or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size showing stable right hepatic lobe segemnt VI hypodense focal lesion measures 1.2x1.2 cm.
- Still noted gall bladder stone.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

CT Scan
[[DOCTYPE:CT_SCAN]]

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ادارة الاشعة

Total DLP (mGy.cm):

Patient Name: Ebtesam Zein Al Abedin Mohamed	Date : 22/07/2024
Exam : C.T CHEST	Age : 56 Year(s)
Medical ID: 431/2022	Pat. ID: 15768

- Stationary ostepenic texture of the examined bones with multiple tiny radiolucent foci.

Conclusion:

- **Follow up study running over all stationary course since the previous study.**

Dr. Ali Elsisy, MSc

Signature Dr. Mohamed Mohsen

24/07/2024 06:19 م

CT Scan
[[DOCTYPE:CT_SCAN]]